

MEDICARE ADVANTAGE BRIEFING

CMS-HCC Model v28: The 2,294 Codes Dropped From Risk Adjustment

How the shift from v24 to v28 is reducing RAF scores and benchmark capitation reimbursements across medical groups.

2,294

ICD-10 Codes Cut

115

v28 HCC Categories

100%

v28 Phase-In by 2026

THE REGULATORY SHIFT

Why CMS Completely Overhauled Risk Adjustment

CMS reclassified the entire Hierarchical Condition Category (HCC) mapping to curb perceived diagnostic upcoding and align weights with actual Medicare Part A & B fee-for-service cost realities.

-6.8% to -14.2%

Projected RAF Score Decline on Unadjusted Practices

Clinics that continue documenting using v24 coding habits will see direct reductions in Medicare Advantage benchmark funding and shared savings distributions.

From 86 to 115 Payment Categories

While CMS increased total HCC categories to 115, it removed 2,294 non-predictive codes and restructured hierarchical constraints across cardiac, renal, and endocrine conditions.

CLINICAL FINANCIAL IMPACT

The 3 Biggest Clinical Hits Under CMS-HCC Model v28

01 Uncomplicated Diabetes (E11.9) Dropped

Previously mapped to HCC 19 (RAF ~0.105). Under v28, diabetes without documented manifestation or chronic complication maps to zero payment HCCs.

02 Angina Pectoris & Peripheral Vascular Disease

Angina pectoris codes (I20.9) stripped from vascular HCCs. Peripheral vascular disease weights reduced by over 40% under the new restructured categories.

03 Protein-Calorie Malnutrition Restrictions

Severe protein-calorie malnutrition criteria heavily constricted to prevent automated EMR templating from generating unverified risk weight.

THE ADAPTATION PLAYBOOK

How Clinicians & Coders Must Adapt Documentation

 Rule 1: Specify End-Organ Manifestations

Never document standalone diabetes or hypertension. Explicitly link diabetic chronic kidney disease (E11.22 + N18.3) or neuropathy (E11.40).

 Rule 2: Enforce MEAT Documentation Criteria

Every chronic condition billed must have documented evidence of Monitor, Evaluate, Assess, or Treat during the face-to-face encounter.

 Rule 3: Run Model Delta Comparisons (v24 vs v28)

Calculate dual-model RAF scores across your entire panel to pinpoint patients experiencing artificial revenue drops.

 Rule 4: Certified Risk Adjustment Coders (CRC)

Deploy AHIMA/AAPC certified CRC coders to conduct pre-encounter chart reviews and identify gaps before claim submission.

TAKE ACTION

Model Your Practice's v24 vs v28 RAF Score Delta

INTERACTIVE BENCHMARK TOOL

Free HCC RAF Calculator & Audit

Test your top ICD-10 diagnosis combinations through our interactive calculator to see exact v24 vs v28 weight changes, demographic adjustments, and revenue projections.

- Instant v24 vs v28 RAF Score Variance Computation
- Specialty risk-adjustment documentation playbooks included

Access free resources:

Calculate HCC RAF Score Delta
aetherahealthcare.com/tools/hcc-raf-calculator



Schedule Risk Adjustment Audit
aetherahealthcare.com/schedule



Request Documentation Guide
aetherahealthcare.com/contact

